

Policy on Registration and Admission

«Hospital Name»

Document control

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Prepared by	Registration In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Every patient who is assessed is registered; every admission is authorised by a doctor; a unique number follows the patient; beds and clinical priority are managed so that the hospital accepts only what it can care for.

1. Purpose

This policy says how «Hospital Name» uses written guidance to register and admit patients (including unidentified patients); generates a unique identification number at the end of registration; accepts patients only when it can provide the required service (with life-saving treatment first in emergency); manages non-availability of beds; and prioritises access according to clinical need.

The chapter intent is that only those patients who can be cared for by the organisation are admitted, and that emergency patients receive life-stabilising treatment and are then either admitted or transferred appropriately.

This policy does not cover which services are offered, transfer-out when services are unavailable, or specific informed-consent procedures for invasive procedures — those are covered in other hospital policies.

2. Scope

This policy applies at every registration and admission point at «Hospital Name»: front office, out-patient registration, emergency, day-care and in-patient admission desks. It binds registration and front-office staff, treating doctors who authorise admission, nurses who receive admitted patients, the Medical Superintendent, department heads, and the Quality Coordinator.

It covers written guidance for registering and admitting patients, including unidentified patients; unique identification numbering at end of registration; accepting patients only when the required service can be provided; managing non-availability of beds; and prioritising access according to clinical need — for out-patients, day-care, in-patients and emergency patients.

Boundaries:

- The service directory used to decide acceptance is covered in other hospital policies.
- Transfer-out when beds or services are unavailable beyond temporary holding is covered in other hospital policies.
- Specific and invasive-procedure consent is covered in other hospital policies. This policy owns that general consent for treatment is obtained at entry and that its scope is defined and explained; general consent must not include invasive procedures that need specific consent.
- Billing owns tariffs; this policy owns explaining financial implications when the desired bed category is unavailable.
- Medical Records owns the unique-number master index in coordination with registration.

3. Policy standards

«Hospital Name» uses written guidance for registering and admitting patients, including unidentified patients. All patients assessed in the hospital are registered. Every admission is authorised by a doctor. A unique identification number is generated at the end of registration and used across the organisation. Patients are accepted only if the organisation can provide the required service; in emergency, life-saving treatment is started before any acceptance decision. Written guidance manages non-availability of beds. Access is prioritised according to clinical need.

«Hospital Name» does not admit without doctor authorisation, does not leave an assessed patient unregistered, and does not refuse life-saving emergency treatment while deciding acceptance.

4. Non-negotiable rules

1. Do not leave an assessed patient unregistered — including unidentified patients, who are registered under the unidentified-patient process.
2. Do not admit a patient without authorisation by a doctor.
3. Do not complete registration without generating the unique identification number at the end of registration, and do not give a second unique number to the same patient.
4. Do not accept a non-emergency patient for a service the hospital cannot provide; in emergency, start life-saving treatment before deciding acceptance.
5. Do not hold a patient on a temporary bed beyond the defined time without a decision to transfer or place; explain financial implications when the desired bed category is unavailable.

6. Do not ignore clinical priority in out-patient or diagnostic queues when a patient needs earlier response.
7. Staff who find registration, unique-number or bed-management rules not followed report it the same shift to the Medical Superintendent or the Quality Coordinator.

5. What we do

5.1 Written guidance for registering and admitting patients

«Hospital Name» keeps written guidance for registration and admission covering out-patients, day-care, in-patients and emergency patients. The guidance includes unidentified patients. All patients who are assessed in the hospital are registered. Government regulations that apply to registration and admission are followed. Documentation is designed to avoid duplication — information once generated is available to departments that need it.

Identity verification at registration is done by government photo ID where available, or two identifiers stated by the patient or accompanying person. All admissions are authorised by a doctor. Additional documentation for foreign nationals is collected as required by applicable rules.

General consent for treatment is obtained when the patient enters the organisation. The organisation defines the scope of that general consent and explains it to the patient and/or family. General consent does not include invasive procedures or other procedures that need specific consent under the PRE consent policy.

5.2 Unique identification number at end of registration

At the end of the patient's first registration interaction with «Hospital Name», a unique identification number is generated. That number identifies the patient across the organisation and supports continuity of care. All hospital records of the patient carry this number.

Unique means a one-time assignment: a patient has only one unique number. For later out-patient or in-patient visits a visit or encounter number may be generated in addition; those numbers are linked to the unique number. Registration staff must not create a second unique number for the same person.

5.3 Accept patients only when the required service can be provided

Registration and admission staff know the services «Hospital Name» can provide (from the service directory) and whom to contact for clarification — the duty doctor, the Head of the relevant department, or the Medical Superintendent.

Patients are accepted only if the organisation can provide the required service. In emergency, life-saving treatment is initiated before any decision about acceptance. If after stabilisation the hospital still cannot provide the needed service, transfer-out procedures apply.

5.4 Managing non-availability of beds

«Hospital Name» maintains a current list of alternate organisations where patients may be directed when beds are unavailable.

If patients are admitted to a temporary holding area, that area has adequate infrastructure to care for them. The guidance defines how long patients may remain on temporary beds before a transfer-out decision is taken — not more than 4 hours, based on The Joint Commission's patient-safety standard for emergency department boarding (Leadership Standard LD.04.03.11, Patient Flow), unless the Medical Superintendent extends in writing for a named patient. This is the organisation's adopted practice; NABH does not mandate a specific timeframe here.

When a bed is not available in the desired bed category or unit, staff manage placement per the guidance and explain the financial implications to the patient and/or family before confirming the alternative category.

5.5 Prioritise access according to clinical needs

Access to healthcare services is prioritised according to the patient's clinical needs in out-patient and diagnostic settings. Patients whose clinical problem warrants an earlier response are identified and seen sooner.

All staff who handle queues and diagnostic flow are oriented to these guidelines. «Hospital Name» uses a visual identification mechanism — a coloured sticker on the file or on the patient clothing — so that all concerned staff can recognise priority patients. The mechanism is defined in writing and taught at induction.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for registration and admission written guidance, bed non-availability rules and clinical-priority rules. Approves alternate-organisation list and temporary-holding time limits.
Registration / front-office staff	Register all assessed patients, including unidentified patients. Generate the unique identification number at end of registration and never duplicate it. Obtain general consent at entry and explain its scope. Apply clinical-priority visual identification in OPD and diagnostics.
Treating doctors	Authorise every admission. Decide acceptance against available services; start life-saving treatment in emergency before acceptance decisions.
Nursing administration / ward nurses	Receive admitted patients; escalate when temporary holding exceeds the defined time.
Medical Records	Maintain the unique-number master index

	linked to visit numbers.
Quality Coordinator	Audits registration, unique-number integrity, bed-holding and priority identification.
Department heads	Clarify service availability for registration staff when contacted.

7. Quality monitoring (RCA → CAPA)

What is monitored:

- Written registration and admission guidance is current and covers unidentified patients, OP/IP/day-care/emergency.
- Sample of admissions shows doctor authorisation and unique identification number on all records.
- No duplicate unique numbers in the sample; visit numbers are linked where used.
- Temporary-holding episodes stay within the defined time; financial explanation for category mismatch is documented where applicable.
- Clinical-priority visual identification is in use in OPD and diagnostics; staff orientation records exist.

Root-cause analysis is required when a duplicate unique number, an unauthorised admission, or a patient remaining in a temporary holding area beyond the defined time limit recurs; the organisation's defined recurrence threshold applies, as documented in the hospital's Root Cause Analysis procedure.

This policy is reviewed annually, and sooner when registration systems or bed capacity change.

8. Training and staff acknowledgement

Registration, front-office, medical records, treating doctors and nurses who admit patients are informed of this policy at induction and once a year after that. Training covers unidentified-patient registration, unique-number rules, general consent scope at entry, bed non-availability, and clinical-priority visual identification.

Staff acknowledgement

I have read the Policy on Registration and Admission of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. Registration in-charge and the Quality Coordinator hold signed acknowledgements.)

9. Distribution

Official master copy: office of the Medical Superintendent, «Hospital Name», with the Quality Coordinator.

Copies issued to: registration; front office; medical records; emergency; day-care; every ward; nursing administration; billing (bed-category financial implications).

The current version is available to all staff at the front-office policy file and, if the hospital keeps an intranet, at staff intranet / policies.

When a new version is issued, take old copies out of use.

10. Abbreviations

Abbreviation	Meaning
AAC	Access, Assessment and Continuity of Care (NABH Hospitals chapter)
CAPA	corrective and preventive action
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	objective element
OP / OPD	out-patient / out-patient department
PRE	Patient Rights and Education (NABH chapter)

11. Traceability to NABH HCO Full Accreditation 6th Edition AAC.2

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
AAC.2.a	Commitment*	The organisation uses written guidance for registering and admitting patients.	Section 3; 5.1	Medical Superintendent (approve guidance); Registration / front-office (execute); Treating doctors (authorise admission)
AAC.2.b	CORE	A unique identification number is generated at the	Section 3; 5.2	Registration / front-office (generate); Medical Records

		end of the registration.		(master index)
AAC.2.c	Commitment	Patients are accepted only if the organisation can provide the required service.	Section 3; 5.3	Registration / front-office (screen); Treating doctors (acceptance and emergency life-saving); Department heads (clarify services)
AAC.2.d	Commitment*	The written guidance also addresses managing patients during non-availability of beds.	Section 3; 5.4	Medical Superintendent (approve guidance and time limits); Nursing administration (temporary holding); Registration (alternate organisations and financial explanation)
AAC.2.e	Commitment*	Access to the healthcare services in the organisation is prioritised according to the clinical needs of the patient.	Section 3; 5.5	Registration / front-office and diagnostic reception (apply priority); Quality Coordinator (audit orientation)

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

AAC.2.a — Written guidance for registering and admitting patients

- Written registration and admission guidance covering OP, day-care, IP, emergency and unidentified patients.
- Sample registration records showing assessed patients registered and general consent obtained at entry with scope explained.
- Admission orders or notes showing doctor authorisation for each admission in the audit sample.

AAC.2.b — Unique identification number generated at end of registration

- Unique-number master index showing one unique number per patient.
- Sample clinical records carrying the unique number across departments.
- Audit log of duplicate-number checks with corrections.

AAC.2.c — Patients accepted only when the required service can be provided

- Current service directory available at registration points.
- Emergency records showing life-saving treatment started before acceptance decisions where applicable.
- Log of non-accepted non-emergency presentations with reason and advice given.

AAC.2.d — Written guidance for non-availability of beds

- Written bed non-availability guidance including temporary holding, time limits and category mismatch.
- Current list of alternate organisations for redirection.
- Temporary-holding log with arrival time, decision time and outcome.
- Documented explanation of financial implications when desired bed category is unavailable.

AAC.2.e — Access prioritised according to clinical needs

- Written clinical-priority guidelines for OPD and diagnostic services.
- Description of the visual identification mechanism (sticker or equivalent) and where it is placed.
- Staff orientation records for queue and diagnostic staff.
- Sample of priority patients identified and seen earlier, with visual marker noted.

13. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Guidebook to Accreditation Standards for Hospitals, 6th Edition — Access, Assessment and Continuity of Care (AAC), standard AAC.2.
- The Joint Commission. (n.d.). Leadership Standard LD.04.03.11 (Patient Flow) — boarding time frames not to exceed 4 hours, as cited in Mohr, N. M., et al. (2020). Boarding of Critically Ill Patients in the Emergency Department. *Critical Care Medicine*, 48(8), 1180-1187. <https://pmc.ncbi.nlm.nih.gov/articles/PMC7365671/> (official standard text is subscription-only; this links to the free, open-access journal article that cites it).

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the

Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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